

RADIOLOGY-STYLE SUMMARY

Cervical Spine MRI Analysis Report

EXAM	TECHNIQUE
MRI cervical spine	Automated analysis of sagittal cervical spine MRI. Research-use structured interpretation only; not a substitute for radiologist review.
CASE ID	SUBMITTED
scan_demo_c5	2026-06-07T14:10:00Z
SOURCE FILE	PATIENT CONTEXT
scan.nii.gz	Male, 42 years, 178 cm
SUMMARY	
4 rows, 3 flagged, 1 syndromes	

FINDINGS

Alignment

Cervical alignment is straightened / low lordosis. C3-C7 Cobb angle measures -2.0 deg. Approximate classification only: derived from C3-C7 Cobb on supine MRI.

Level-Specific Findings

At C5, functional canal / dural-sac AP minimum (SCT) 9.8 mm (stenosis_provisional); space available for the cord (canal AP - cord AP) 2.7 mm (high_risk); cord T2-hyperintensity screen (myelomalacia) 1.0 present (signal_anomaly_present).

Cord / Syndrome Pattern

C5: pattern consistent with possible cervical myelopathy; clinical correlation required

IMPRESSION

1. C5: pattern consistent with possible cervical myelopathy; clinical correlation required
2. C5: functional canal / dural-sac AP minimum (SCT) 9.8 mm (stenosis_provisional).
3. C5: space available for the cord (canal AP - cord AP) 2.7 mm (high_risk).
4. C5: cord T2-hyperintensity screen (myelomalacia) 1.0 present (signal_anomaly_present).
5. Alignment: straightened / low lordosis; C3-C7 Cobb angle -2.0 deg.

DISCLAIMERS

- This is a research tool, not a medical device. Not for clinical diagnosis.
- Measurements acquired on supine MRI; functional radiographs may differ.